

Clinical features

Symptoms

Most patients are asymptomatic and only notice the presence of visible warts. When symptoms occur, they may include **itching**, **bleeding**, or **dyspareunia**.

Physical signs

Typical anogenital warts appear as superficial papular lesions, 1–5 mm in diameter. They may be flat or pedunculated, and can occur as solitary or multiple lesions. Multiple warts may coalesce into larger plaques, particularly in immunosuppressed individuals or when left untreated. Warts usually match the surrounding skin tone but may be more pigmented.

Warts can develop on any anogenital skin surface, but are commonly found at sites prone to trauma during sexual intercourse—such as the **preputial cavity in men** and the **posterior fourchette in women**. Anal canal warts are more frequent in men who have sex with men (MSM) who report condomless anal intercourse or other practices involving anal penetration. Perianal warts, however, are common in both sexes and may occur even without a history of anal intercourse.

Complications

Psychosexual impact

Genital warts can significantly affect sexual health and quality of life. Patients may experience **anxiety**, **guilt**, **anger**, and **reduced self-esteem**. Concerns about **fertility** and **cancer risk** are also common.

Precancer and cancer

Anogenital warts are benign and do not undergo neoplastic transformation. However, **premalignant lesions** (such as VIN, AIN, and PIN) or **malignancies** may coexist with or develop within wart-like lesions, or may be misdiagnosed as warts. Clinical features suggesting neoplastic change include **bleeding, ulceration, or palpable dermal infiltration**. In such cases, **urgent biopsy or specialist referral** is indicated.

Bowenoid papulosis is characterized by reddish-brown lesions associated with oncogenic HPV types and is considered part of the spectrum of anogenital intraepithelial neoplasia.

A rare complication of HPV 6/11 infection is **giant condyloma acuminatum (Buschke-Löwenstein tumor)**, a form of **verrucous carcinoma** that infiltrates underlying tissues. Management typically involves **surgical resection**, with possible adjuvant use of **chemoradiotherapy, topical retinoids, or imiquimod**. These cases require **specialist surgical and oncological care**.

Diagnosis

- Use a **good light source** during examination (1D).
- **Magnification** (e.g., hand lens or colposcope) may aid detection of small lesions (2D).
- Examine the **urethral meatus** routinely (1D).
- In women with anogenital warts, **vaginal warts** occur in approximately **15%** and **cervical warts** in about **6%**. Offer a **speculum examination** if cervical or vaginal involvement is suspected—such as when lesions are present at the introitus or if the patient reports awareness of internal lesions (1D).
- Perform **perianal inspection** in all patients at initial assessment or if symptoms (e.g., anal lesions or irritation) are reported (1D).

- If anal canal warts are suspected (e.g., external lesions extending internally, anal bleeding, or discharge), offer **digital rectal examination** and **proctoscopy** (1D).
- **Biopsy** is not required for typical warts but should be performed if there is **diagnostic uncertainty** or **suspicion of precancer or cancer** (1D).